

OTHER GRADE PAPER

Rating certainty of evidence when the target threshold is a value-based threshold and the point estimate is close to that threshold

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Abstract

Objectives: It is crucial, when using Grading of Recommendations Assessment, Development and Evaluation (GRADE), that authors are clear about what it is in which they are rating the certainty of evidence (ie, the target of certainty of evidence rating). This article addresses challenges GRADE users face in choosing a target of certainty rating for any value-based threshold (ie, little or no difference; an important effect; small, moderate or large difference) when the point estimate is close to that threshold.

Study Design and Setting: Using an iterative process, authors generated and refined possible solutions to the identified challenges.

Results: The challenges GRADE users face when point estimates prove close to the initially chosen value-based threshold include (1) the intuitive interpretation of the point estimate is similar whichever side of the threshold the point estimate lies on but the formal interpretation differs greatly and (2) rating down for imprecision due to the confidence interval (CI) crossing the threshold when the CI is very narrow. This paper provides four possible solutions. The first is continuing to rate certainty in an important or unimportant effect and rating down for imprecision when the CI crosses the MID. The second is to consider uncertainty around the MID, rating certainty in relation to a range of plausible MIDs (ie, a range formed by the smallest plausible MID and the largest plausible MID), and rate down for imprecision when the CI crosses either plausibility bound. The third solution is considering both the null and moderate effect thresholds, rating certainty in a trivial or small effect, and rating down for imprecision when the CI crosses either threshold. The fourth solution is using more than one of the above solutions and presenting two or more certainty of evidence ratings.

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Conclusions: GRADE users should note these challenges and apply their chosen solution to transparently determine the target of their certainty rating. © 2026 The Author(s). Published by Elsevier Inc. This is an open access article under the CC BY license (<http://creativecommons.org/licenses/by/4.0/>).

Keywords: GRADE; Target of certainty of evidence rating; Imprecision; Value-based threshold; Minimal important difference

Plain Language Summary

When point estimates prove to be close to the initially chosen value-based threshold, to decide on what it is in which GRADE users rate their certainty (ie, the target of certainty rating), GRADE users face challenges. This paper provides four possible solutions related to the minimal important difference (MID) that are also applicable to other value-based thresholds. (1) Continuing to rate certainty in an important or unimportant effect and rate down for imprecision when the CI crosses the MID; (2) Considering a plausible range of MIDs, rating certainty in an effect that is close to the MID, and rating down for imprecision when the CI crosses either or both plausibility bounds; (3) Considering both the null and moderate effect thresholds, rating certainty in a trivial or small effect, and rating down for imprecision when the CI crosses either threshold; and (4) presenting more than one certainty of evidence rating. GRADE users may note these challenges and consider applying the solutions to transparently determine the target of their certainty rating and to make certainty assessments accordingly.

1. Introduction: target of certainty rating and the limitations of using thresholds

Prior to 2017, GRADE guidance specified that one rates certainty with respect to the point estimate. In keeping with this target of certainty rating, guidance for rating down for imprecision emphasized, for systematic reviews, the optimal information size, and for recommendations, a decision threshold that considered overall benefits, harms, and burdens. In 2017, GRADE guidance offered a fundamentally modified concept of certainty of evidence and suggested, for both systematic reviews and guideline recommendations, rating certainty that the true effect lies on one side of a threshold or within a particular range (ie, target of certainty rating) [1,2]. Core GRADE, a series of papers published in the BMJ that presents the key elements of GRADE, re-enforces this modification [3,4].

Deciding on the target of certainty rating involves, first, setting one or more thresholds. Possible thresholds include the null, the minimal important difference (MID or small effect threshold, defined as the smallest change that patients perceive as important benefit or harm), the moderate effect threshold, and the large effect threshold. Often, systematic review authors using GRADE specify the null as their threshold and initially rate certainty in a nonzero effect [5]. Alternatively, and also frequently, review authors and guideline developers choose the MID as their threshold and rate certainty in an important or unimportant effect. Much less frequently, they choose multiple thresholds. Whatever their choice, if the confidence interval (CI) crosses the chosen threshold or thresholds, they rate down for imprecision.

Using all four thresholds allows specification of ranges of little to no, small, moderate, and large effects [1,2]. The location of the point estimate in relation to a chosen threshold or range determines the target of certainty rating [1,2]. For instance, if the point estimate is in the small effect range (situations A and B in Fig 1), GRADE users using only the MID threshold will rate certainty in an

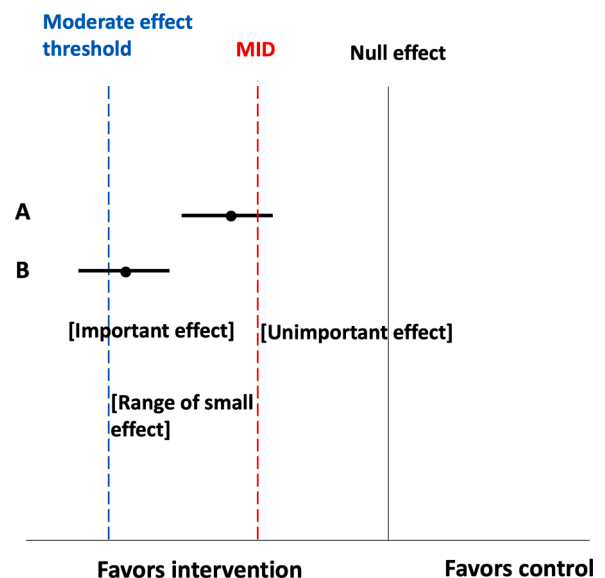


Figure 1. When using MID as a threshold, GRADE users will rate their certainty in an important effect and rate down for imprecision if the CI crosses the MID. When using a range of small effect (formed by the MID and the moderate effect threshold), they will rate their certainty in a small but important effect and rate down for imprecision if the CI crosses either the MID or the moderate effect threshold. CI, confidence interval; MID, minimal important difference.

What is new?

Key findings

- The two challenges in deciding on the target of certainty rating include rating certainty in an effect that falls above or below a threshold when the most likely inference is that the effect is very close to that threshold; and rating down for imprecision due to an overlap with the threshold when the CI is very narrow.
- This paper provides four possible solutions for considering target of certainty rating when the point estimate is close to an initial value-based threshold, in particular the minimal important difference (MID).
- Solution 1 retains the simplicity of rating certainty in an important or unimportant effect at the cost of failing to directly address two challenges (ie, rating certainty in an effect that falls above or below a threshold when the most likely inference is that the effect is very close to that threshold; and rating down for imprecision due to an overlap with the threshold when the CI is very narrow).
- Solution 2 addresses both challenges but introduces complexity by requiring specification of the largest and smallest plausible MIDs for which, in most cases, there is very little evidence.
- Solution 3 suggests rating certainty in a small or moderate effect, directly addressing the two challenges but resulting in a conclusion that the truth represents a trivial or small effect, a wide range that might not be useful for the end-users of the systematic review.
- Solution 4 suggests simultaneously using two or more of the above approaches offering more information but adding further complexity.

What this adds to what is known?

- By considering the difference between value-based thresholds and the null effect threshold, this paper for the first time addresses the problems in deciding on target of certainty of evidence rating when the point estimate is near a value-based threshold.

What is the implication and what should change now?

- To decide the target of certainty of evidence rating when the point estimate is close to an initially chosen value-based threshold, GRADE users should note two fundamental challenges.

- Considering the feasibility and usefulness, GRADE users will choose one of the four possible solutions and transparently decide on their target of certainty rating.

important effect. If the CI crosses the MID, they will rate down their certainty of evidence for imprecision (situation A in Fig 1). If not, they will not rate down (situation B in Fig 1). When using the MID and the moderate effect thresholds that form a range of small effect, they will rate their certainty in a small effect. If the CI crosses either the MID or the moderate effect threshold (situations A and B in Fig 1), they will rate down for imprecision.

Clinical epidemiology and evidence-based medicine rely heavily on use of thresholds, and making inferences based on whether point estimates like above or below the chosen threshold. This simplifies matters greatly, but introduces a ubiquitous problem: point estimates slightly above or below the threshold provide essentially the same information but very different interpretations. There is no ideal solution to this dilemma; practitioners typically ignore the issue, considering only whether point estimates are on one or the other side of the threshold and rarely focusing on an inference that the truth is near the chosen threshold. This paper addresses the problem in relation to GRADE certainty of evidence ratings.

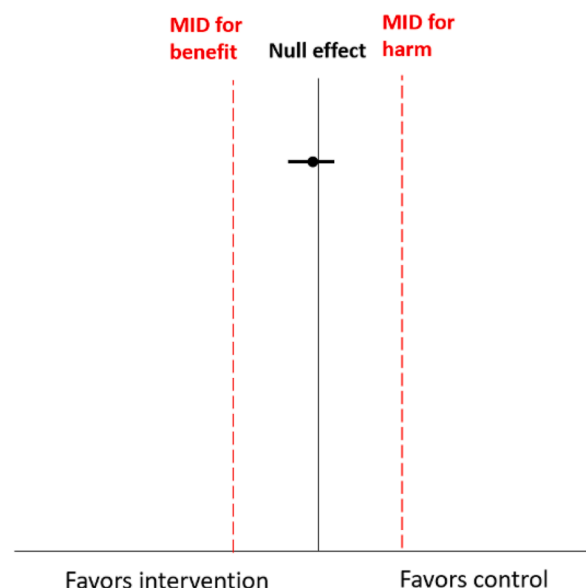


Figure 2. When the point estimate is close to the initial chosen null effect threshold, rating certainty in a nonzero effect is misleading, and when the CI is very narrow rating down for imprecision is problematic. CI, confidence interval.

2. Challenges in deciding on the target of certainty rating when a point estimate is close to the initial chosen threshold: the null threshold versus value-based threshold

In a previous paper, we introduced two challenges that can occur when GRADE users initially choose the null effect threshold and the point estimate falls close to the null (Fig 2) [6]. The first is that rating certainty in a nonzero effect is misleading (ie, a benefit or a harm) when the most reasonable inference is that the effect is unimportant (ie, little or no effect). The second follows from the logic of using the CI to judge imprecision: wide CIs warrant rating down for imprecision and narrow CIs do not. The challenge lies in deciding how wide is too wide: In cases like Figure 2, rating down for imprecision when the CI is very narrow is problematic. The solution we provided was to modify the target of certainty rating from a nonzero effect to little or no effect. The MIDs for benefit and harm define the boundaries of little or no effect [6].

The two problems described reflect the limitations of the binary nature of the null effect threshold but are relevant to use of any threshold. In contrast to choice of the null as the threshold, which involves no value judgment, GRADE users choosing the MID, the moderate effect or large effect

threshold must, in deciding where to place the threshold, make a value judgment. We therefore refer to these three thresholds as value-based.

Consider a hypothetical systematic review evaluating a new intervention X for patients at risk of myocardial infarction. The user of this systematic review is interested in knowing whether intervention X results in an important reduction in myocardial infarction. In response, systematic reviewers choose a difference of 15 in 1000 (1.5%) as the MID and thus as their threshold for rating their certainty of evidence.

In situation A of Figure 3, the meta-analysis shows that intervention X yields 1.6 fewer myocardial infarctions per 100 adults, with a 95% CI from 2.4 fewer to 0.8 fewer per 100 patients. Because the point estimate falls above the MID threshold, reviewers rate their certainty in intervention X having an important effect. As the CI crosses the MID threshold, they rate down for imprecision.

In situation B of Figure 3, because the point estimate, 1.4 fewer per 100 patients, falls below the MID, reviewers rate their certainty in intervention X having an unimportant effect on the risk of myocardial infarction. Again, because the CI crosses the MID threshold, they rate down for imprecision. However, given both point estimates represent a similar magnitude of effect, rating certainty in one being

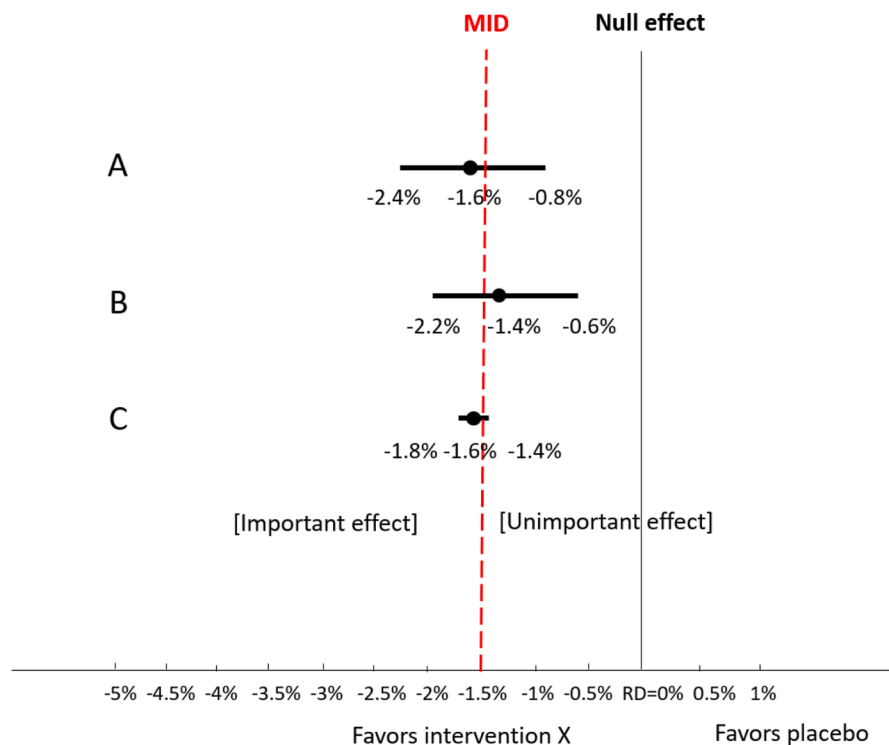


Figure 3. Hypothetical systematic review of intervention X for reducing myocardial infarction in patients at risk of myocardial infarction. In situation A and C, according to current GRADE guidance, because the point estimate falls above the MID threshold, reviewers would rate their certainty in intervention X having an important effect and rate down for imprecision. In situation B, because the point estimate falls below the MID, reviewers would rate their certainty in an unimportant effect and rate down for imprecision. MID, minimal important difference.

an important effect and the other being unimportant ignores the fact that they convey essentially the same information.

Another potential challenge in such situations, illustrated in Figure 3C, exists when the point estimate is near the MID and the CI is very narrow. Here, if reviewers adhere to the original MID threshold (1.5%), because the CI crosses the MID, they will rate down for imprecision. However, the intuitive inference when the CI is very narrow is that there is very little imprecision.

In this paper, we review, with respect to the value-based thresholds, the two key challenges that arise when the point estimate is near the threshold. Because of either variability in or uncertainty around patient values, the value-based thresholds are often associated with considerable uncertainty. The null effect threshold (eg, risk difference = 0, or relative risk = 1) is, in contrast, not associated with any uncertainty. This key difference results in a wider range of options if GRADE users have chosen one or more value-based thresholds than if they have chosen the null as the threshold.

We provide alternatives for addressing the challenges in deciding on the target of certainty rating when the point estimate is close to a value-based threshold. As the MID is of particular interest in both systematic reviews and guidelines [4,7,8], we discuss the challenges and solutions related to the MID. These solutions are also applicable for deciding target of certainty rating when GRADE users initially choose a moderate or large effect threshold and the point estimate falls close to the chosen threshold.

3. Possible solutions for deciding the target of certainty rating, making imprecision judgments, and communicating conclusions: an example from the literature for binary outcome

Consider the following example from a systematic review of low-dose computed tomography (LDCT) vs. no screening or any non-LDCT intervention on all-cause mortality in adults that includes a meta-analysis of 91,107 participants from eight randomized controlled trials [9]. Results suggest that LDCT screening reduces all-cause mortality by 0.44% with a 95% CI from 0.80% fewer to 0.09% fewer (Fig 4).

Assume the reviewers were interested in whether an important effect is present and initially set an MID threshold at a difference of 0.5%. As the point estimate (a reduction of 0.44%) is close to the MID threshold, and the CI around the pooled estimate crosses the threshold but is very narrow, the reviewer is confronted with the challenges we have outlined. For certainty of evidence rating, the reviewers rated down one level for risk of bias and had no concerns regarding indirectness or publication bias. How should they have addressed imprecision?

3.1. Solution 1: continuing to rate certainty in an important or unimportant effect

When the point estimate is near the MID, GRADE users might choose to continue rating certainty in relation to the MID and rate down for imprecision if the CI crosses the MID.

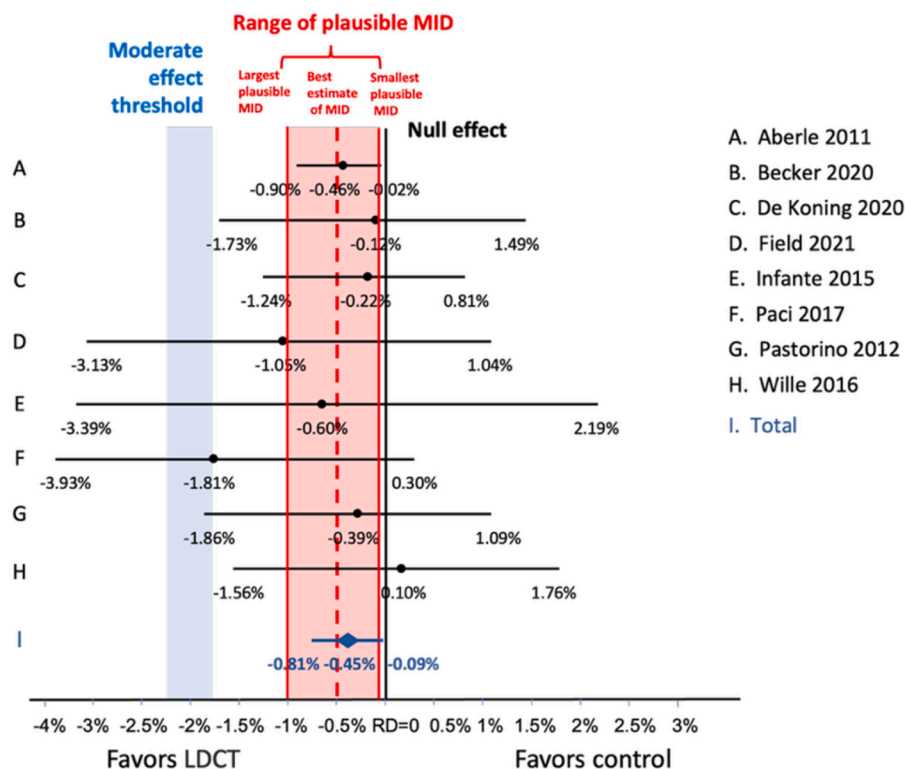


Figure 4. Meta-analysis of low-dose computed tomography (LDCT) vs. no screening or any non-LDCT intervention on all-cause mortality.

In the LDCT screening example, if the reviewers continue to choose the MID as the threshold, they will rate their certainty in LDCT having an unimportant effect on mortality (Fig 4). As the CI crosses the MID, they will rate down for imprecision. They will not rate down for inconsistency because although the point estimates from the individual trials fall on different sides of the MID, they are all very close to the threshold. Together with the concern regarding risk of bias, they will infer low certainty. They would then conclude that LDCT may have an unimportant effect on mortality.

The merit of this solution is its simplicity. GRADE users can continue with their initial choice of threshold and rate certainty accordingly. This is consistent with the priorities of Core GRADE: introducing complexity only if failure to do so will result in important misleading inferences of the magnitude and certainty of treatment effects, or important misleading recommendations [3,10]. The shortcomings, however, are first that users may conclude greater certainty in the inference of an unimportant effect than is warranted. In this case, with limitations and in both risk of bias and precision and consequently only low certainty evidence, this is not a problem. Had reviewers not rated down for risk of bias and concluded moderate certainty evidence, it might have been. Second, if one applies guidance in a rigid way, the approach does not address the challenge of rating down for imprecision when the CI is narrow (although doing so ameliorates the problem of higher certainty in an unimportant effect than is warranted).

3.2. Solution 2: revising the target of certainty of evidence rating to an effect that is close to the MID

Due to the variation in values among patients and the usual lack of high quality of evidence regarding patient values, setting MIDs typically involves considerable uncertainty [11–13]. When the point estimate proves close to the MID, another solution for deciding on the target of the certainty rating is to consider uncertainty around the MID and to rate certainty in relation to a range of plausible MIDs: the smallest plausible MID and the largest plausible MID.

Following this reasoning, GRADE users would revise their target of rating certainty, moving from an important or unimportant effect to an effect that is close to the MID. The imprecision judgment would then depend on whether the CI crosses the threshold of the smallest and/or largest plausible MID. If the CI crosses either or both plausibility bounds, GRADE users will rate down for imprecision. If not, they will not rate down. For inconsistency judgment, if the effect estimates from individual trials fall both within and outside of the plausible range of MID, GRADE users will consider rating down for inconsistency.

In the LDCT screening example, our hypothetical reviewers initially set a single MID threshold (ie, a difference of 0.5 in 100) and found a point estimate close to that threshold. Given the uncertainty around the MID threshold, using this second solution, they will consider a range of

plausible MIDs that reflects their uncertainty around the MID. If the reviewers consider the largest plausible MID as a difference of 1 in 100 and the smallest plausible MID as a difference of 0.1 in 100 or 1 in 1000 (Fig 4), as the pooled point estimate falls within that range (ie, a range between 0.1 and 1 in 100), the reviewers will revise their target of certainty rating to an effect that is close to the MID (ie, the effect is on the border of being important). Further, as one end of the CI (0.09 fewer in 100) falls outside the range of plausible MIDs, they will rate down for imprecision. As the effect estimates of only two out of eight individual trials slightly falls outside the range of plausible MIDs, the reviewers will probably not rate down for inconsistency. Thus, they will judge certainty as moderate and conclude that the reduction of LDCT on lung cancer-related mortality is probably close to the MID (Fig 4).

Explicitly specifying the smallest and largest plausible MID is always challenging. GRADE users might not necessarily adduce specific single values for these two thresholds. All they need to do is to judge whether the point estimate falls within the range of plausible MIDs. For imprecision, they will judge whether both boundaries of CI falls within the range, and for inconsistency judgment whether the effect estimates from most of the individual studies fall within that range.

The merit of solution 2 is that, compared with solution 1, it addresses the two challenges by offering a target of certainty rating that more closely fits with intuition of the most likely effect, as well as a more appropriate imprecision judgment. The shortcoming is that it is considerably more complex. It requires considering the largest and smallest plausible MIDs which in most cases have very little supporting evidence. Appendix 1 discusses the choice of range of plausible MIDs. It is thus speculative in nature, and many reviewers are likely to find its complexity challenging, and potentially prohibitively challenging. It is not, therefore, appropriate for Core GRADE.

3.3. Solution 3: revising the target of certainty of evidence rating to a trivial or small effect

Another possible solution GRADE users might consider is to modify the threshold of interest from the MID to the two thresholds adjacent to the MID, the null effect and the moderate effect thresholds, and rate their certainty in a trivial to small effect.

If GRADE users choose this solution, they will judge whether the boundaries of the CI overlap with the null effect threshold and the moderate effect threshold. If either of the boundaries is crossed, they will rate down for imprecision. If not, they will not rate down. For inconsistency, if the individual effect estimates from primary studies fall within the range of trivial or small effect, they will not rate down for inconsistency.

Returning to the LDCT screening example, if the reviewers choose solution 3, they will rate their certainty in

a small or moderate effect. If they consider that a mortality reduction of 0.81% is less than the moderate effect, they will not rate down for imprecision, regardless of whether the moderate effect threshold was 0.9%, 1.0% or 1.2% (Fig 4). If they consider the effect estimates from individual primary trials also fell within that range, they will not rate down for inconsistency. Accordingly, they will rate down only for risk of bias and conclude that the effect of LDCT screening on reducing mortality is probably a trivial or small effect with moderate certainty of evidence.

The merit of this solution is that it addresses the two challenges and, compared with solution 2, requires specifying only a single specific moderate effect. Even easier, they need not specify a single specific value for the moderate effect, but only judge whether the upper boundary of the CI represents a small or moderate effect. The limitation of this solution is that a conclusion regarding certainty in whether the true effect is trivial or small effect might not be very useful for the end-users of the systematic review who are required to make a decision about recommending for or against the intervention. If the effect is trivial or small, they may conclude that issues other than benefit (convenience, cost) would decide on the optimal CT approach. If small but important, they would suggest in favor of LDCT.

3.4. Solution 4: presenting more than one certainty of evidence rating

In the LDCT screening example, if the reviewers choose to present more than one certainty of evidence rating, for example, both solution 1 and solution 3, they will conclude that the effect of LDCT screening on reducing mortality is probably a trivial or small effect (moderate certainty of evidence) but they have only low or very low certainty in whether that effect is important. If the reviewers choose solution 2 and solution 3, they will conclude that LDCT screening probably has a trivial or small effect and that effect is probably close to the MID.

The merit of presenting more than one certainty of evidence ratings is that the reviewers can provide more information to the user of the review. The limitation is that it will add even more complexity to the exercise.

Whatever they decide, GRADE users should, before they review the results from the meta-analysis, clarify in their protocol all the strategies that they are going to consider when they review the results.

4. An example from the literature for continuous outcome

We have presented four possible solutions for deciding on the target of certainty rating when the point estimate is close to the MID threshold and used an example from the literature for a binary outcome to illustrate these solutions. In this section, we present an example for a

continuous outcome and clarify how these solutions work in this context. Compared with dichotomous outcomes, MID choice for continuous outcomes might be more evidence-based. Fortunately, for a large number of patient reported outcomes, excellent evidence regarding MIDs is available, and thus choosing an MID or considering the range of plausible MIDs might be less challenge [14].

Consider a systematic review of intense pulsed light therapy for patients with dry eye symptoms [15]. The meta-analysis of 595 patients from seven randomized controlled trials yields a reduction of 7.36 points on Ocular Surface Disease Index (OSDI) instrument (a validated instrument for measuring dry eye disease with a range of 0 to 100, higher scores representing worse symptoms) with a CI ranging from a reduction of 12.18 points to a reduction of 2.53 points (Fig 5).

The authors of this systematic review were interested in whether an important effect exists. Informed by a study that generated the MID threshold for the OSDI instrument [16], the reviewers set the MID at a difference of 7 on the 0 to 100 scale. For certainty of evidence rating, they had concerns regarding risk of bias (rated down one level), and no concerns regarding indirectness or publication bias.

The reviewers adopted solution 1, and as the point estimate (a reduction of 7.36) is greater than the MID, they rated certainty that the intense-pulsed light therapy has an important effect on relieving dry eye symptoms. Because the CI crosses the MID, they rated down for imprecision. Moreover, because the effect estimates from primary trials fall on different sides of the MID, they rated down for inconsistency (Fig 5). Then, they concluded very low certainty of evidence, using wording conveying that it is unclear whether intense pulsed light therapy has an important effect (Table 1).

Had the conclusion struck the reviewers as too low a rating, using the gestalt approach recommended by Core GRADE [3,4], they might have considered rating down one level for imprecision and inconsistency together and concluded low certainty evidence of an important effect.

If reviewers choose solution 2, they will rate their certainty in relation to the range of plausible MIDs. Without considering the exact boundary of the range, reviewers can judge that the point estimate falls within that range (ie, the point estimate represents an effect that is close to the MID). They will thus rate their certainty that the effect of intense pulsed light therapy on relieving dry eye symptoms (measured by OSDI scale) is close to the MID (Fig 5). If the reviewers consider that the lower boundary of the CI (a reduction of 2.53 points on OSDI scale) represents an unimportant effect, or that 12.18 clearly represents an important effect, they will rate down for imprecision. Given that the point estimates lie on different sides of the MID, they will rate down for inconsistency. Given serious risk of bias, imprecision, and inconsistency concerns they will conclude that there is very low certainty on whether the intervention has an effect near the MID. Using a gestalt

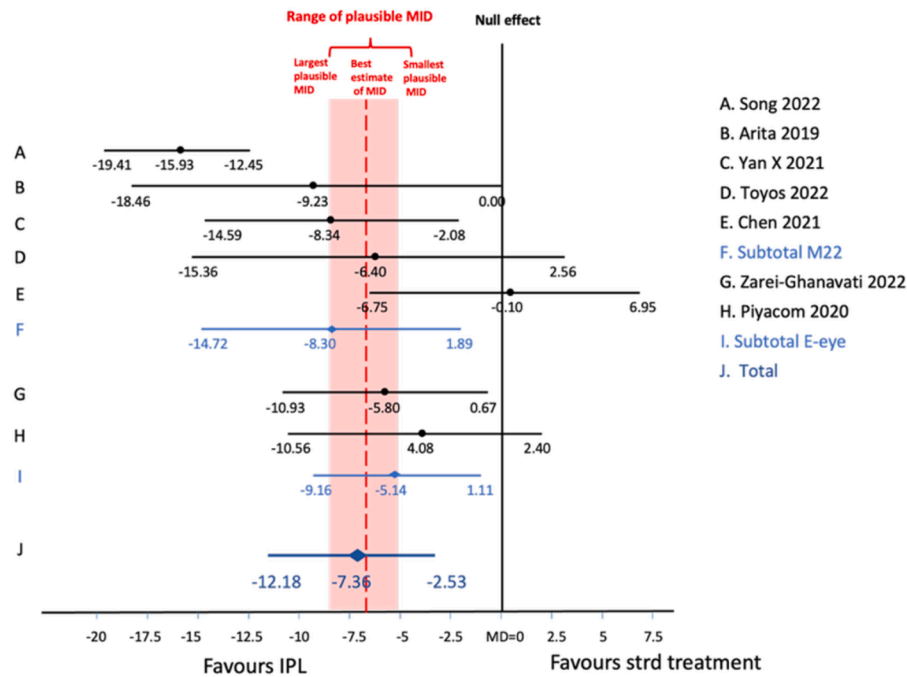


Figure 5. Meta-analysis of intense pulsed light therapy vs. standard therapy for patients with dry eye symptoms.

approach, they might consider rating down one level for imprecision and inconsistency together and would conclude low certainty evidence that the truth lies near the MID.

If the reviewers choose solution 3 and consider the point estimate of 7.36 points reduction on OSDI falls below a

moderate effect threshold (which they certainly should), they will rate their certainty that the effect of intense pulsed light therapy on relieving dry eye symptoms is trivial or small. If the reviewers consider the upper boundary of the CI (a reduction of 12.18 points) falls below the moderate effect

Table 1. Target of certainty ratings, certainty assessments and plain language summaries for solution 1-3 in the meta-analysis of therapy for patients with dry eye symptoms

Solution	Target of certainty rating	GRADE certainty of evidence rating						Plain language summary
		Risk of bias	Imprecision	Inconsistency	Indirectness	Publication bias	Certainty of evidence	
Solution 1	An important effect	Rate down 1 level	Rate down 1 level	Rate down 1 level	No concern	No concern	Very low or low on basis of risk of bias and concerns with imprecision and inconsistency	It is unclear whether intense pulsed light therapy has an important effect. Alternatively, therapy may have an important effect.
Solution 2	An effect close to MID	Rate down 1 level	Rate down 1 level	Rate down 1 level	No concern	No concern	Very low or low on basis of risk of bias, and concerns with imprecision and inconsistency	It is unclear whether intense pulsed light therapy has an important effect. Alternatively, therapy may have an effect that is close to the MID.
Solution 3	A trivial or small effect	Rate down 1 level	No concern	No concern	No concern	No concern	Moderate	Intense pulsed light therapy probably has either a trivial or small effect on relieving dry eye symptoms.

MID, minimal important difference.

threshold and the other CI of 2.53 clearly represents a trivial effect, they will not rate down for imprecision. They will conclude that intense pulsed light therapy probably has a trivial or small effect on relieving dry eye symptoms.

If the reviewers choose solution 4, they will have moderate certainty, concluding that the effect is probably trivial or small, but it would be unclear whether that effect is an important effect or an effect that is close to the MID.

5. The choice between solutions

We have a great deal of experience using our first suggested approach but very little using the second and third solutions. Nevertheless, our description of the strengths and limitations of each approach will assist GRADE users in choosing their desired approach. Our lack of experience with the second and third approaches prevents us from offering detailed additional considerations. We do have additional thoughts that we present in the following paragraphs.

Teams who put a priority on simplicity and straightforwardness, and thus an inclination to follow Core GRADE guidance, will choose the first approach. A further consideration is whether the outcome under consideration is likely to be key to the direction of the recommendation. If this is the case and the point estimate is near the MID threshold, the recommendation is certain to be conditional. Which side of the MID the point estimate falls and the consequent wording that the effect possibly or probably represents an important or unimportant effect may help the panel to decide on a recommendation for or against the intervention.

The second approach, because it requires specifying the range in which the MID or other thresholds might lie, is complex and challenging. It also has the limitation that the associated plain language summary does not provide help on whether to make a conditional recommendation for or against the intervention. Thus, it might be a good choice if a team has the interest and resources to take on the challenge of specifying the range of plausible MIDs or other thresholds, and some other benefit—or more likely harm—outcome will determine the direction of the recommendation. For instance, if the magnitude of the benefit outcome lies near the MID, and the intervention has an appreciable probability of important harm and then a recommendation in favor of the comparator might be a good choice. In such a situation, rating certainty that the true effect is near the MID might be very helpful.

Authors may have the expertise and resources to organize, for example, a focus group with patients. They would then gain an understanding of their views regarding the range of plausible MIDs. Alternatively, they may have discussions within the panel complemented by a formal survey of panel views [17,18]. The focus group involves both additional complexity and resource expenditure, the survey additional complexity.

The third approach is the one that, compared with the first approach, adds the least additional complexity and challenges in decision-making. The only additional value-based decision required if the effect is near the MID is specifying, or at least developing a general idea, of where the moderate effect threshold lies. The approach is the most likely to avoid rating down for imprecision, but that is because it rates certainty over a wide range of possible effects from trivial (immediately adjacent to the null) to almost moderate in magnitude (immediately adjacent to the moderate effect threshold). Such a broad range of possibilities might be least problematic if the outcome under consideration is not key to recommendation.

The fourth approach provides more information about certainty of evidence ratings which might be useful. For example, in the intense-pulsed light therapy example, if the reviewers chose solution 4, they will conclude that intense pulsed light therapy probably has a trivial or small effect on relieving dry eye symptoms, but it is unclear whether that effect is important. Such conclusions provide more information than only applying one solution.

6. Conclusion

This paper introduces two challenges that occur when GRADE users initially rate the certainty of evidence in relation to a value-based threshold and the point estimate falls close to that threshold. We provide four possible solutions and illustrate their application with two examples from the literature including review of the associated challenges with each approach. In particular, rating certainty that the truth lies in the range in which the threshold may lie entails the difficult challenge of defining that range, and presenting more than one certainty rating adds complexity.

CRedit authorship contribution statement

Linan Zeng: Writing — original draft, Resources, Project administration, Methodology, Funding acquisition, Data curation, Conceptualization. **Monica Hulcrantz:** Writing — review & editing, Methodology, Data curation. **Romina Brignardello:** Writing — review & editing, Methodology, Formal analysis. **David Rind:** Writing — review & editing, Methodology, Formal analysis. **Mengting Yang:** Writing — original draft, Visualization, Investigation. **Tong He:** Visualization, Investigation. **Reem A. Mustafa:** Writing — review & editing. **Ariel Izcovich:** Writing — review & editing. **M. Hassan Murad:** Writing — review & editing. **Arnav Agarwal:** Writing — review & editing. **Farid Foroutan:** Writing — review & editing. **Lingli Zhang:** Writing — review & editing. **Gordon H. Guyatt:** Writing — review & editing, Supervision, Formal analysis, Conceptualization.

Declaration of competing interest

All authors declare that they do not have financial relationships with any organizations that might have an interest in the submitted work. Linan Zeng, Monica Hultcrantz, Romina Brignardello, David Rind, Reem A. Mustafa, Ariel Iz-covich, M. Hassan Murad, Arnav Agarwal, Farid Foroutan, and Gordon Guyatt are members of GRADE working group (GWG). This project is GRADE-related but not a GWG endorsed paper.

Supplementary data

Supplementary data related to this article can be found at <https://doi.org/10.1016/j.jclinepi.2026.112344>.

Data availability

Data will be made available on request.

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